

ACTIVE RECOVERY PHYSICAL THERAPY

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INITIAL EVALUATION

Patient:	David Park
DOB:	9/24/1986
ID:	PT-3349
Eval Date:	7/8/2024
Therapist:	Hailey Chen, DPT, OCS
Referring MD:	Tehrani, Reza, MD
Diagnosis:	M54.16 – Radiculopathy, lumbar region; s/p MRI 7/1/24 demonstrating right L4-L5 disc protrusion with L5 nerve root impingement
Frequency Rx:	2x/week x 6 weeks

SUBJECTIVE

Patient reports right low back pain with intermittent radiation to right posterior thigh and occasionally to lateral calf. Symptom duration ~ 8 weeks. Onset associated with lifting boxes during household move. Symptoms worse with prolonged sitting (his work) and improved with walking. Pain rating: 3/10 at best (early morning), 6/10 at worst (mid-afternoon at desk). Sleep is affected approximately 2-3 nights per week.

Patient goals: return to running (was running 3x/week pre-injury), tolerate full work day at desk without pain spike, return to weekend hiking with partner.

Has not done formal PT before. Highly motivated and compliant by report.

OBJECTIVE

Posture:	Mild forward head, increased thoracic kyphosis (occupational). Lumbar lordosis preserved.
ROM:	Lumbar flexion limited to ~50% with pain reproduction at end-range. Extension full and pain-free. Side-bending full bilaterally.
Strength:	Hip extensors 5/5 B Hip abductors 4+/5 R, 5/5 L Quads 5/5 B Tibialis ant 5/5 B EHL 4+/5 R, 5/5 L Gastrocs 5/5 B (Mild weakness on R consistent with L5 distribution.)
Sensation:	Subjective decreased sensation lateral R calf; objective testing equivocal.
Reflexes:	See ortho note 6/18 — R Achilles 1+, L Achilles 2+.
Special tests:	R SLR + at ~55 deg with reproduction of radicular sx. Slump test + on R.
Functional:	Sit-to-stand difficult, uses arms for assist. Single-leg balance R 12 sec, L 28 sec.

ASSESSMENT

38 yo M with right L5 radiculopathy 2/2 right L4-L5 disc protrusion confirmed on MRI. Presents with mild motor involvement (hip abduction, EHL) on R side, consistent with L5 distribution, but no significant strength deficit. Functional limitations are primarily related to prolonged sitting tolerance, which is a critical demand for his occupation. Good rehab candidate: motivated, no red flags, anatomy is manageable, no surgical urgency.

PLAN

PT 2x/week x 6 weeks (12 visits total). Treatment to include:

- Mechanical traction trial sessions 1-3
- McKenzie-based directional preference assessment
- Core stabilization progression (transverse abdominis, multifidus activation)
- Hip strengthening (gluteus medius particularly given the R weakness)
- Neural mobilization techniques for L5 nerve root
- Postural re-education and ergonomic counseling for desk work
- Progressive return to running program over weeks 4-6 if symptoms allow

Goals at 6 weeks:

1. Pain rating < 2/10 at rest, < 4/10 at end-of-day.
2. Negative SLR or symptom reproduction beyond 70 degrees.
3. Tolerate 2 hours continuous sitting without pain escalation > 2 points.
4. Initiate run-walk progression by week 5.

Will communicate with Dr. Tehrani after visit 6 (mid-point) re: progress.

— Hailey Chen, DPT, OCS
Active Recovery Physical Therapy